

US lawsuits expose how trust-based sharing of health data heightens privacy risks

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By [Maria Dinzeo](#)

Lawsuits over the alleged misuse of patient records are spotlighting privacy gaps in the trust-based systems underpinning US health data exchanges.

Telehealth company GuardDog recently made the unusual confession that it had spent much of its two-year existence accessing sensitive health data under false pretenses.

In a court filing, GuardDog said it had infiltrated national health record exchange called Carequality not for the purpose of providing patient treatment, but to gather data to information to law firms hunting for mass-tort clients.

The admission was the latest twist in a lawsuit brought by several hospitals and Epic Systems, one of the largest electronic health record software companies in the United States, against more than a dozen healthcare entities accused of committing “clinical camouflage”—exploiting their access to two health information exchanges to extract valuable patient data (see [here](#)).

“Epic is acting to protect patient privacy and uphold the law. Medical records are deeply personal and exploiting them is wrong. We, along with the providers joining us in this action, won’t look the other way when it happens,” the company told MLex in an email.

The case raises the question of whether for health records have outpaced the mechanisms designed to protect patients’ privacy, and whether trust alone is enough to safeguard their most sensitive data.

— Vulnerability in the system —

The exchange frameworks, Carequality and the Trusted Exchange Framework and Common Agreement, or TEFCA, were built to let doctors share records for treatment. Use has surged in recent years. According to the Department of Health and Human Services, roughly 10 million documents had been exchanged through TEFCA before 2025; by the end of 2025, that figure had climbed to 464 million. Carequality says its network enables the exchange of more than 1.2 billion clinical documents each month.

But access is highly dependent on participants accurately representing themselves and their purpose when requesting patient records. This relatively low bar for entry creates a vulnerability in the system that is ripe for abuse. And patients may never know their data was accessed, let alone misused.

Further complicating matters, the complaint alleges that the frameworks themselves do not independently verify whether intermediaries have adequately vetted the entities they connect. This is by design, as both Carequality and TEFCA are run by the non profit organization Sequoia Project, which sets the rules that govern the frameworks but doesn’t have the staff to independently review every participant that requests access to records.

Instead, it delegates that responsibility to third-party gatekeepers. Disputes around concerns over unauthorized access must be resolved through a process that is largely opaque, Epic Systems claims, “where vendor confidentiality is valued above protecting the privacy rights of patients.”

The Sequoia Project declined to speak with MLex, but said through a spokesperson that “Transparency and integrity are the pillars of a trusted health data exchange,” and that “TEFCA Governing Bodies and Qualified Health Information Networks work closely to ensure that every participant remains a responsible steward of sensitive patient data. Under TEFCA, QHINs are responsible for their participants, with ongoing monitoring by the entire community.”

Enter Health Gorilla, an intermediary that connects healthcare providers to the exchanges and is responsible for vetting participants seeking access to Carequality and TEFCA.

According to the complaint, Health Gorilla’s vetting process was minimal to nonexistent, a claim that it categorically

rejects.

"Epic's complaint presents unfounded and wholly misleading allegations about Health Gorilla's role in connecting health care providers with needed data about patients," Health Gorilla CEO Bob Watson said in a statement. "Health Gorilla has long operated in strict conformance with all applicable laws, governance requirements, and industry norms."

He added that on learning of Epic Systems' claims, Health Gorilla immediately suspended the offending providers and began investigating the use of the data.

Defendants like medical records company LlamaLabs have also pushed back against claims of wrongdoing. "Patient privacy has always come first. We have never sold, stolen, or misused patient data, and we never will," its CEO Shere Saidon said in a statement.

—A wider problem —

Epic's allegations also expose a broader gap in US health privacy law. While patients often assume their medical data remains protected under the Health Insurance Portability and Accountability Act, or HIPAA, the law applies primarily to certain healthcare organizations rather than the data itself.

A recent report by the Electronic Privacy Information Center argues that modern health data systems increasingly allow sensitive information to flow beyond those entities, where it can be used in ways patients do not expect and cannot control.

"Our health privacy standards need to be updated to meet the moment," the report says. "Technology has shifted dramatically over the last thirty years, but we don't have legal protections to keep pace. Instead, our health data is increasingly being harvested, sold, and used beyond our control, creating a health privacy crisis."

Sara Geoghegan, lead author of the report, told MLex that Epic Systems' lawsuit only underscores the need for stronger health data protections. "If we have robust rules that regulate the collection, transfer and retention of our personal information and that prohibit that information from being used in out of context ways that we would not expect, and if we had strong enforcement with teeth of those rules and regulations to protect our privacy, then actors like this, these practices, would be less prevalent," she said.

— Downstream effects —

Concerns about privacy gaps are not limited to advocates. In a letter to the Sequoia Project, dozens of hospitals and health systems warned of "a clear pattern of bad actors improperly obtaining patients' medical information" and called for stronger oversight of the frameworks.

"To protect patients' privacy, there is an urgent need for the frameworks to implement centralized vetting, onboarding, and monitoring controls," their letter says. "Details on exchange activity must be made publicly available, and when potential privacy issues are identified they must receive timely, effective, and transparent resolutions.

They are right to be concerned. Since March, at least a dozen putative class actions have been filed in courts all over the US after hospitals began notifying consumers patient data had been accessed under false pretenses.

The cases could put pressure on the frameworks to finally address the structural tension in a system where medical records can be shared seamlessly, but access controls rely too heavily on trust.

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